

العمر(تاريخ الميلاد) : 03-03-1972 المهنة : ربه منزل

التوقيع: (_____)

التعليق على التشخيص : -----

تتعمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

رئيس
المجلس الطبي المتخصص

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Name: إيمان صلاح امين حماد محمد National ID: 27203030102088	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Goserelin (Zoladex) subcutaneous

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

Patient's Name: Iman Salah Amin Hamad Mohammed
Date: 2026-05-11
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

091.

MULTISLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH CONTRAST

TECHNIQUE: Multi slice axial CT images of the chest, abdomen and pelvis were obtained followed by multiplanar reconstruction of the source images. I.V contrast administration.

FINDINGS:

****Compared to previous study dated on 27-12-2025:**

CECT CHEST:

- Stationary course of,
 - Left MRM with clear operative bed.
 - Right upper lung lobe, anterior segment subpleural nodule measures 4 mm.
- No cavitary lesions or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

CECT ABDOMEN AND PELVIS:

- Stationary course as regard,
 - The liver is average in size showing homogenous CT density with right hepatic lobe subcapsular calcified lesion is seen at segment VII measures 12 x 10 mm.
 - Bulky left adrenal gland.
- No biliary radicles dilatation.
- The gall bladder is not visualized.
- Normal CT appearance of the spleen, pancreas and both kidneys.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- No abdominal or pelvic pathologically enlarged lymph nodes.
- No free collection.

THE SCANNED SKELETON:

- The scanned skeleton shows stationary course of heterogenous texture of the examined bones with widely spread sclerotic lesions seen at the axial and appendicular skeleton.
- L5-S1 spondylolisthesis.

Patient's Name: Iman Salah Amin Hamad Mohammed
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- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- *A follow up status running a stationary course since the last study dated on 27-12-2025.*

Dr.Heba El Balshy.MD

*Lecturer and Consultant of Diagnostic and Interventional Radiology,
NLI-Menoufia University*

Dr.Heba Elbalshyp

Dr.Heba Elbalshy

Name: إيمان صلاح امين حماد		
MR. : 5210		
Detailed Medical Report		
		السيد
تحية طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient has metastatic breast cancer with documented spread to the lung and bone. Treatment has been ongoing with Faslodex and Ibrance since 2023. The patient continues to have metastatic disease to the lung and bone and is receiving Faslodex, Ibrance, Zoladex, and Zometa, which were initiated in 2023. The most recent computed tomography performed in December 2025 demonstrated stable disease. The patient received Zoladex in February 2026 and the last dose of Zometa was administered in October 2025. Laboratory studies are adequate, and the current regimen includes Ibrance, Faslodex, and Zometa. The patient presents with metastatic breast cancer involving bone and lung, treated with Ibrance and Faslodex since April 2023. Laboratory results are acceptable, and the patient remains on Ibrance and Faslodex. The patient has a history of left breast cancer treated with neoadjuvant chemotherapy and left modified radical mastectomy, with subsequent recurrence and metastatic spread to lung and bone. Current management includes Zometa, Zoladex, Faslodex, and Ibrance, with a recent CT in May 2026 showing stable disease. The treatment plan continues with Zometa, Zoladex, Faslodex, and Ibrance.		
تحريراً في ,		
	الوقت :	التاريخ : 19-09-2026
	Ali Adel	الطبيب المعالج :
يعتمد ,		

بالتأكيد..



اسم الفرع: حمية الزيتون
مدير الوحدة: د خالد عبد الله
طبيب الوحدة: د. نيفين فاروق

Visit Number: 42126506800

Patient Name: ايمان صلاح امين

Age / Sex: 54 Year / Female

Referred By: Prof : محمد اسماعيل

Client Name: 6343

Registered 09-05-2026 12:26:01

Collected 09-05-2026 12:27:50

Authenticated 09-05-2026 19:09:06

Reported 09-05-2026 20:47:24

HEMATOLOGY REPORT

TEST NAME	RESULT	UNIT	REFERENCE RANGE
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Complete Blood Picture

Haemoglobin (EDTA Blood)	L 11.0	g/dL	11.5 - 15.5
Haematocrit (PCV)	L 33.5	%	36 - 45
RBCs Count (EDTA Blood)	4.25	Millions / cmm	4 - 5.2
MCV	L 78.8	fL	80 - 100
MCH	L 25.9	pg	27 - 33
MCHC	32.8	g/dL	31 - 37
RDW-CV	H 23.3	%	11.5 - 15
Platelet Count	262	Thousands / cmm	150 - 450
Total Leucocytic Count (EDTA Blood)	5.1	Thousands / cmm	4 - 11

Percent Values

Neutrophils	26.6	%
Lymphocytes	60.0	%
Monocytes	10.8	%
Eosinophils	1.2	%
Basophils	1.4	%

Absolute Values

1.35	x10 ⁹ /L	2 - 7
3.04	x10 ⁹ /L	1 - 4.8
0.55	x10 ⁹ /L	0.2 - 1
0.06	x10 ⁹ /L	0.1 - 0.45
0.07	x10 ⁹ /L	0 - 0.1

Other Cells

Comment:

Mild hypochromic microcytic anemia with RBCs anisocytosis. Serum iron, TIBC and ferritin are recommended., Relative lymphocytosis., Relative monocytosis., Absolute neutropenia. Follow up is recommended.

Dr. Khaled Abdellah
Clinical pathology consultant

Abdellah

1-1

خصم 30% على جميع أنواع الأشعة لفترة محدودة لا يسري مع التعاقبات ولا أي خصومات أخرى



19911

Lab Test
[[DOCTYPE:LAB_TEST]]

بالتأكيد..



اسم الفرع: حمية الزيتون
مدير الوحدة: ايد منال كمال
طبيب الوحدة: د. منال حامد

Visit Number:	42126506800	Registered	09-05-2026 12:26:01
Patient Name:	انسان صلاح امين	Collected	09-05-2026 12:27:50
Age / Sex:	54 Year / Female	Authenticated	09-05-2026 18:37:18
Referred By:	Prof: محمد اسماعيل	Printed	09-05-2026 20:47:21
Client Name:	6343		

TEST NAME	RESULT	UNIT	REFERENCE RANGE
Liver Function Tests			
Total Bilirubin	0.19	mg/dL	0 - 1.2
Direct Bilirubin	0.09	mg/dL	0 - 0.3
Indirect Bilirubin	0.10	mg/dL	0 - 0.9
SGPT (ALT)	12	U/L	0 - 33
SGOT (AST)	17	U/L	0 - 32

Kidney Function Tests			
Ionized Calcium	1.18	mmol/L	1.12 - 1.32
Serum Creatinine	0.74	mg/dL	0.5 - 1.1
Serum Urea	24	mg/dL	13 - 43
Serum Uric Acid	H 6.2	mg/dL	2.4 - 5.7
Serum Calcium	8.7	mg/dL	8.6 - 10.2

Dr. Manal Kamal
Professor of Clinical & Chemical Pathology,
Faculty of Medicine, Cairo University

Manal Kamal

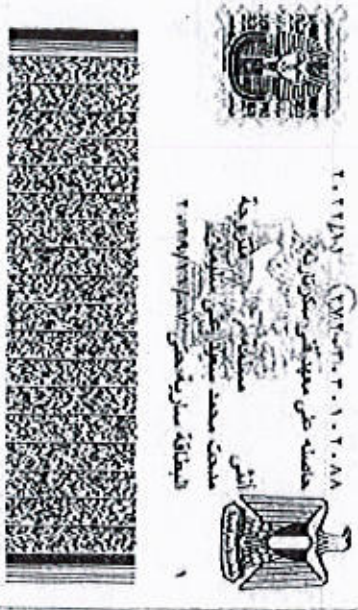
1-1

خصم 30% على جميع أنواع الأشعة لفترة محدودة لا يسري مع التعاقدات ولا أي خصومات أخرى



19911

Lab Test
[[DOCTYPE:LAB_TEST]]



HEALTH INSURANCE ORGANIZATION
CENTER DEPARTMENT OF
INFORMATION AND DECISION SUPPORT



الهيئة العامة للتأمين الصحي
الإدارة المركزية لتكنولوجيا المعلومات
ودعم القرار

جهة الإصدار: لمرع القاهرة

تاريخ الإصدار: 2023-10-17

الى من يهمه الأمر

الهيئة العامة للتأمين الصحي تعمل من خلال قوانين منظمة للانتفاع الخاضعين لقانون التأمين الاجتماعي رقم 79 لسنة 1975 المعدل بالقانون 25 لسنة 1977 "القوى العاملة والمعاشات والارامل" وكذلك قانون 99 لسنة 1992 الخاص بطلاب المدارس و قانون رقم 86 لسنة 2012 " الاطفال دون السن المدرسي" وقانون رقم 23 لسنة 2012 الخاص بالمرأة المعيلة .
وطبقا لقاعدة البيانات المرسلة من التأمينات الاجتماعية
وطبقا لقاعدة البيانات المرسلة من الضمان الاجتماعي
والبحث تبين أن

السيد (ة) ايمان صلاح امين حماد محمد بطاقة رقم 27203030102088

وطبقا الى خطاب من التأمينات الاجتماعية يفيد عدم الانتفاع
يعتبر من الفئات غير المنتفعة بالتأمين الصحي صدر هذا الخطاب دون اى مسؤولية على الهيئة
العامة للتأمين الصحي حال خضوع المواطن لقانون التأمين الصحي الشامل
وشكرا

الموظف المختص

سماح محمود

تاريخ الطباعة: 2023-10-17

إفادة مركز نظم المعلومات
بمقتضى



مستمل 1082221

الخطاب ساري لمدة ثلاثة أشهر من تاريخ الإصدار إلا في حالة تغيير حالة المواطن من قبل الهيئة العامة للتأمينات الاجتماعية أو
وزارة التضامن الاجتماعي أو وزارة التربية والتعليم

الإدارة المركزية لتكنولوجيا المعلومات، ودعم القرار

Name : EMAN SALAH AMEIN

Date : 27-12-2025 11:25

Branch : Portable

Medical No. : GRI-42973

Ref.Physician Prof :

Ins.Company :

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS
REVEALED: -**

COMPARED TO STUDY DATED 11/8/2025, THE CURRENT STDY REVELED

1-Chest:

- Left MRM with clear operative bed
- Still seen prominent left axillary LN likely non specific
- Still seen right upper lobe anterior segment subpleural nodule measures 4mm
- No sizable pulmonary mases or consolidation
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Still seen enlarged left thyroid lobe with heterogenous texture

2-Abdomen and pelvis:

- The liver is average in size with homogenous CT density, stationary course of the small subcapsular marginally calcified isodense lesion at segment VII
- Still seen associated subcapsular calcifications likely post granulomatous
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Cholecystectomy clip
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and right supra-renal.
- Still seen bulky left adrenal gland
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- IUCD is in place

3-The scanned skeleton:

- The scanned skeleton shows rather stationary course of heterogenous texture of the examined bones with widely spread sclerotic osseous lesions of axial and appendicular skeletons
- No vertebral structural collapse or extra-osseous soft tissue component.
- Dorso-lumbar spondylosis.
- Still seen L5-S1 1ST degree spondylolisthesis

OPINION:

Stationary course as regarding previous study

DR/NERMEEN RAMADAN.MD



Patient Name: Eman Salah Ameen Hammad Mohamed	Date : 03/12/2024
Exam : MAMMOGRAPHY BOTH	Age : 52 Year(s)
Medical ID: 2823/2023	Pat. ID: 42973

**DIGITAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY
ULTRASOUND**

Indication:

Patient is 52 years old and is seen for follow-up after left MRM

Report:

- Left MRM:
 - Operative bed shows no mass or collection
 - No regional lymphadenopathy.
- Right Breast:
 - Predominant Fatty parenchyma (ACR A)
 - No suspicious micro calcifications or architectural distortion detected.
 - Scattered foci of benign calcifications
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.

Opinion:

- Post left MRM status showing no evidence of recurrence (BIRADS 2)
- Benign findings of right breast (BIRADS 2)

Best regards,
Dr Reem Mahmoud
Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCI, Cairo University
03/12/2024 1:34 PM



Age / Gender 52 Years - Female

Registered Date 2023-05-23 12:49 م

Reporting Date 2023-05-31 09:32 م

Visit No. 23-2908

Patient Name

إيمان صلاح أمين حماد

PATHOLOGY REPORT

Specimen :

Technique :

Immunostaining effectuated on 4 um thick paraffin sections from the paraffin block, processed in Benchmark GX (Ventana) and stained by primary antibodies against DAB as chromogen and hematoxylin as counter stain The sections were treated against ER, PR, Her23neu, Ki67.

Result :

- ER: Show faint nuclear staining of 2% of neoplastic cells (Allred score 3/8).
- PR: Show faint nuclear staining of 2% of neoplastic cells (Allred score 3/8).
- Her2neu: Show negative staining of neoplastic cells (score 0).
- Ki67: Show positive staining of 5% of neoplastic cells.

Salsabil Mahmoud

Signature

Prof. Dr. Naglaa Samir.
Dr. Mariam EL Rashedy.
29/5/2023

09/03/2024, 12:31

CODE 12 - N.sing_A MDT form Hama - Namia_E MDT form Hama

President's Program for Women's Health
Ministry of Health and Populationالبرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

4- Patient ID:

1- Name: **أحمد محمد محمد**

Sex:

MDT form

3- Medical numbers

Female

83154726

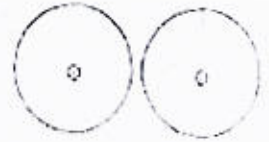
Old Med:

Old Med:

2023-2023

2- ID number: 27203030102088

Patient Age: 52 Y



Family history

None

Phone

Phone

0123795387

BMI

BMI

REASON OF MDT

REASON OF MDT

Menopausal status

None

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT

Comorbidity

None

Menopausal status

Menopausal status

Initial STAGE

Initial STAGE

09/03/2024

Breast Imaging:

Clinical data

Mets Workup:

ER

ER

PR

3/8

Her 2 Neu

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Sonomammography

PATIENT HISTORY

LT BREAST (2nd) received NACT followed by LT MRM --> ad --> port FOLLOWED BY A1
from 9/2022 till 4/2023

at cap 4/2023: pulmonary nodules + bone mets

started ibranex + faslodex from 4/2023 till present

(at cap 2/2024 --> SD)

for adding zometa only

SHE REPORTED RESOLUTION OF SKIN NODELES AFTER START IBRANCE

MDT OUTCOME & PLAN

FOR CONT CDK 4B INH -FASLODEX

ZOMETA 3 MON

MEDICAL ONCOLOGY

SIGNATURE

SURGERY

RADIOLOGY

PATHOLOGY

Psychiatric

Nutrition

MDT Form

[[DOCTYPE:MDT_FORM]]

Imaging report



كايرو سكين
CAIRO SCAN
Radiology & Labs

Name : Mrs. Iman Salah Amin Hammad Mohamed Ref. Phy. Ahmed Zohdy
Date : 12/04/2025
Room Name : Gamma-HE Code : R2375475

Tc-99m MDP DUAL PHASE BONE SCAN:

PROCEDURE:

- Early blood pool and delayed osseous phase whole body scans were obtained 5 minutes and 3 hours respectively post tracer injection.

FINDINGS:-

- No abnormal focal active or cold osseous lesions could be detected all over the scanned skeleton to account for serious osseous lesions including metastases.
- Degenerative changes at multiple spine levels are noted.
- Osteoarthritic changes around both shoulders, and knees are also observed.

OPINION:

The skeleton is free from any scintigraphically detected serious osseous lesions including metastases.

Best regards
Dr. Esraa El-Kholy, MD.
Assist. Professor of Nuclear medicine (NCI)
Cairo University